

Senate File 231

S-3014

1 Amend Senate File 231 as follows:

2 1. By striking everything after the enacting clause and
3 inserting:

4 <Section 1. Section 514F.8, Code 2025, is amended by adding
5 the following new subsections:

6 NEW SUBSECTION. 1A. a. A utilization review organization
7 shall respond to a request for prior authorization from a
8 health care provider as follows:

9 (1) Within forty-eight hours after receipt for urgent
10 requests.

11 (2) Within ten calendar days after receipt for nonurgent
12 requests.

13 (3) Within fifteen calendar days after receipt for
14 nonurgent requests if there are complex or unique circumstances
15 or the utilization review organization is experiencing an
16 unusually high volume of prior authorization requests.

17 b. Within twenty-four hours after receipt of a prior
18 authorization request, the utilization review organization
19 shall notify the health care provider of, or make available to
20 the health care provider, a receipt for the request for prior
21 authorization.

22 NEW SUBSECTION. 2A. A utilization review organization
23 shall, at least annually, review all health care services for
24 which the health benefit plan requires prior authorization and
25 shall eliminate prior authorization requirements for health
26 care services for which prior authorization requests are
27 routinely approved with such frequency as to demonstrate that
28 the prior authorization requirement does not promote health
29 care quality, or reduce health care spending, to a degree
30 sufficient to justify the health benefit plan's administrative
31 costs to require the prior authorization.

32 NEW SUBSECTION. 3A. Complaints regarding a utilization
33 review organization's compliance with this chapter may be
34 directed to the insurance division. The insurance division
35 shall notify a utilization review organization of all

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(amending this SF 231 to CONFORM to HF 303)

1 complaints regarding the utilization review organization's
2 noncompliance with this chapter. All complaints received
3 pursuant to this subsection shall not be considered public
4 records for purposes of chapter 22.

5 Sec. 2. PRIOR AUTHORIZATION EXEMPTION PROGRAM.

6 1. On or before January 15, 2026, all health carriers
7 that deliver, issue for delivery, continue, or renew a health
8 benefit plan in this state on or after January 1, 2026, and
9 that require prior authorizations, shall implement a pilot
10 program that exempts a subset of participating health care
11 providers, at least some of whom shall be primary health care
12 providers, from certain prior authorization requirements.

13 2. Each health carrier shall make available on the health
14 carrier's internet site for each health benefit plan that the
15 health carrier delivers, issues for delivery, continues, or
16 renews in this state, details about the health benefit plan's
17 prior authorization exemption program, including all of the
18 following information:

19 a. The health carrier's criteria for a health care provider
20 to qualify for the exemption program.

21 b. The health care services that are exempt from prior
22 authorization requirements for health care providers who
23 qualify under paragraph "a".

24 c. The estimated number of health care providers who are
25 eligible for the program, including the health care providers'
26 specialties, and the percentage of the health care providers
27 that are primary care providers.

28 d. Contact information for the health benefit plan for
29 consumers and health care providers to contact the health
30 benefit plan about the exemption program, or about a health
31 care provider's eligibility for the exemption program.

32 3. On or before January 15, 2027, each health carrier
33 required to implement a prior authorization exemption
34 program pursuant to subsection 1 shall submit a report to the
35 commissioner of insurance that contains all of the following:

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1 a. The results of the exemption program, including an
2 analysis of the costs and savings of the exemption program.
3 b. The health benefit plan's recommendations for continuing
4 or expanding the exemption program.
5 c. Feedback received by each health benefit plan from
6 health care providers and other interested parties regarding
7 the exemption program.
8 d. An assessment of the administrative costs incurred by
9 each of the health carrier's health benefit plans to administer
10 and implement prior authorization requirements under the
11 exemption program.>
12 2. Title page, by striking lines 1 through 2 and inserting
13 <An Act relating to prior authorizations and exemptions by
14 health benefit plans and utilization review organizations.>

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